Autism

Inclusion Toolkit

Symptoms

- Difficulty with eye contact, social cues, or small talk.
- Strong need for routine or repetitive movements (stimming).
- Over or under-sensitive to noise, light, or textures.

Age & Gender

- Signs often appear by age 2 or 3, but diagnosis can happen at any age.
- Diagnosed more often in males.
- Females are frequently missed because they are better at masking symptoms.

Common myths

- People with autism feel empathy, but may show it differently.
- It’s a lifelong condition and not just for children.
- Not everyone is a genius. Abilities vary widely.

Diagnosis

- Diagnosis is based on observing behaviour and developmental history.
- Usually involves paediatricians, psychologists, and speech therapists.

Professional support

- Speech and Occupational Therapy help with daily living skills.
- There's no medication for autism itself.
- Peer support groups are vital for mental health.

How to help

- Be direct and literal. Avoid sarcasm or hints.
- Lower lights and noise to reduce sensory overwhelm.
- Allow extra time for them to process information and respond.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.

ADHD

Inclusion Toolkit

Symptoms

- Easily distracted, forgetful, or difficulty managing time, focus, and tasks.
- Fidgeting, inability to sit still, or a sense of inner restlessness.
- Interrupting others, difficulty waiting one's turn, or acting without thinking.

Age & Gender

- Symptoms usually appear before age 12, but persist into adulthood.
- Diagnosed more often in males.
- Females are often missed as they tend to have inattentive type ADHD, rather than hyperactive-impulsive type.

Common myths

- It's not 'bad behaviour' or lack of willpower.
- It's not just a childhood phase. Many adults live with undiagnosed ADHD.
- It's not caused by bad parenting, too much sugar, or screen time.

Diagnosis

- Requires a referral from a GP to a specialist (usually a psychiatrist).
- The assessment involves interviews, questionnaires, and reviewing childhood history to see if symptoms impact daily life.

Professional support

- Medication can help improve focus and reduce impulsivity.
- Cognitive Behavioural Therapy (CBT) or ADHD coaching helps with organisation and emotional regulation.

How to help

- Give clear, written instructions rather than long verbal lists.
- Allow headphones or fidget tools to support those with sensory needs.
- Permit movement breaks, and try to avoid punishing timekeeping errors.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.

OCD

Inclusion Toolkit

Symptoms

- Unwanted, intrusive thoughts causing anxiety.
- Anxiety is relieved through repetitive physical or purely mental actions.
- The relief is temporary, then the anxiety cycle returns.

Age & Gender

- Can start in childhood, but normally begins in late teens or early adulthood.
- Affects males and females roughly equally.
- The average diagnosis occurs 9 to 13 years after initial symptoms start.

Common myths

- It’s a serious condition, not just liking things neat.
- People can’t simply choose to stop.
- People with OCD often know their fears are irrational but still feel the anxiety.

Diagnosis

- GP refers to a specialist mental health team for a detailed assessment.
- Doctors use medical criteria to identify obsessions and compulsions.
- Diagnosed when symptoms significantly impact daily life.

Professional support

- CBT with exposure therapy helps face fears without rituals.
- Antidepressants called selective serotonin reuptake inhibitors can reduce symptom intensity.
- Peer support groups help reduce isolation.

How to help

- Do not criticise or rush them during a ritual.
- Do not join in with their compulsions or offer reassurance.
- Keep a calm atmosphere to help lower general anxiety levels.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.

Dyslexia

Inclusion Toolkit

Symptoms

- Difficulty learning to read, spelling, and working out what written words say.
- Trouble hearing or changing the sounds in words.
- Challenges with sequencing, directions, and memory.

Age & Gender

- Often identified in primary school when children begin to learn to read and write.
- Historically diagnosed more in males, but current research suggests it affects all genders equally.

Common myths

- Dyslexia is a language processing difference, not a visual problem (seeing letters backwards).
- It has absolutely no connection to low intelligence or laziness.

Diagnosis

- Carried out by a specialist assessor or an Educational Psychologist.
- Assessments test memory, speed, and the ability to process word sounds.

Professional support

- Assistive technology like text-to-speech software, screen readers, or coloured overlays (see-through coloured sheets placed over text).
- Structured teaching that uses several senses at once, like sight, sound, and touch, helps improve reading and writing skills.

How to help

- Use plain, sans-serif fonts and pastel backgrounds, not bright white.
- Give verbal cues or diagrams rather than long text.
- Allow extra time for reading and processing.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.

Dyspraxia

Inclusion Toolkit

Symptoms

- Poor balance, coordination, and general clumsiness.
- Difficulty with fine motor skills like handwriting or tying shoelaces.
- Trouble planning and organising physical movements.

Age & Gender

- Symptoms usually appear in early childhood.
- Diagnosed more often in males.
- Females are often missed as avoiding active play or having better handwriting can mask issues.

Common myths

- It’s not just being 'clumsy' or lazy.
- Children will not simply grow out of it.
- It doesn’t only affect movement. It often affects organisation and memory too.

Diagnosis

- Assessment by a paediatrician, occupational therapist, or physiotherapist.
- Tests compare motor skills with what’s usual for the person’s age.

Professional support

- Occupational Therapy helps improve daily living skills.
- Physiotherapy helps improve core strength and balance.
- Adaptations like using a laptop can replace handwriting.

How to help

- Break complex physical tasks into smaller steps.
- Allow extra time for movements like dressing or eating.
- Avoid criticism for messiness or dropping things.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.

Dyscalculia

Inclusion Toolkit

Symptoms

- Difficulty understanding number concepts and arithmetic.
- Trouble telling time, counting change, or estimating quantities.
- High anxiety when faced with mathematical tasks.

Age & Gender

- Signs appear early, often when learning to count or recognise numbers.
- Research suggests it affects males and females equally.

Common myths

- It’s not just being 'bad at maths' or lacking effort.
- It’s a neurological difference, not a sign of low intelligence.
- It’s not rare, just less frequently diagnosed than dyslexia.

Diagnosis

- Assessment by a specialist teacher or Educational Psychologist.
- Standardised tests compare how well the person understands and works with numbers with what’s usual for their age.

Professional support

- Teaching that uses several senses at once, with physical objects like counters.
- Technological aids like calculators or voice-activated apps.
- Focus on practical maths needed for daily life (for example, money, time).

How to help

- Provide reference charts or visual aids for numbers.
- Allow extra time for any task involving data or figures.
- Avoid asking them sudden mental maths questions in front of others.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.

Tourette's Syndrome

Inclusion Toolkit

Symptoms

- Involuntary movements like blinking, shrugging, or head jerking (motor tics).
- Involuntary sounds like grunting, throat clearing, or words (vocal tics).
- An uncomfortable physical urge or sensation before the tic happens.

Age & Gender

- Symptoms usually appear between ages 5 and 9.
- Males account for 75% to 80% of diagnoses.

Common myths

- Most people do not swear (coprolalia affects only about 10% of cases).
- Tics are not done for attention or bad behaviour.
- People cannot simply stop their tics.

Diagnosis

- Diagnosed when a person has multiple motor tics and at least one vocal tic.
- Symptoms must have lasted for more than a year.

Professional support

- Comprehensive Behavioural Intervention for Tics teaches strategies to manage tic urges.
- Medication may be used if tics cause pain, injury, or severe distress.
- Support groups help reduce stigma and improve self-esteem.

How to help

- Ignore the tics completely. Do not stare, laugh, or comment.
- Do not tell them to stop or ask them to be quiet.
- Help reduce stress, as strong emotions can increase tics.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.

Anxiety

Inclusion Toolkit

Symptoms

- Constant feeling of dread, racing thoughts, or feeling tense and unable to relax.
- Heart palpitations, sweating, shaking, or shortness of breath.
- Avoiding situations or places to escape the worry.

Age & Gender

- Can develop at any age, though frequently starts in adolescence.
- Females are roughly twice as likely to be diagnosed as males.

Common myths

- It's a medical condition, not drama or attention-seeking.
- People can't just choose to stop feeling anxious or stop worrying.
- Panic attacks feel life-threatening. They aren’t an overreaction.

Diagnosis

- GP or psychiatrist assessment using questionnaires.
- Generalised anxiety disorder is diagnosed when uncontrollable worry persists for 6 months and impacts daily life.

Professional support

- CBT helps change negative thought patterns.
- Medication can manage physical symptoms.
- Mindfulness and exercise support self-care.

How to help

- Listen without judging or saying 'just calm down'.
- Ask what helps them during a panic attack before one happens.
- Understand that plans may change last minute if they feel overwhelmed.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.

Dysgraphia

Inclusion Toolkit

Symptoms

- Handwriting is often illegible and poorly spaced.
- Holding a pen causes physical pain or cramping.
- Written work is noticeably weaker than their strong verbal skills.

Age & Gender

- It is usually identified in early childhood when learning to write.
- It affects all genders but is historically diagnosed more in males.

Common myths

- It is a neurological difference in how the brain controls writing, not just 'messy handwriting' or laziness.
- It does not mean a person has low intelligence.
- They can have excellent physical coordination for everything else.

Diagnosis

- Diagnosed by an Educational Psychologist or Occupational Therapist.
- Assessments test fine motor skills and compare verbal ability to written output.

Professional support

- There is no medication for dysgraphia.
- Occupational therapy improves hand strength and pencil grip.
- Peer support helps individuals share practical technology workarounds.

How to help

- Allow the use of typing or speech-to-text software.
- Provide extra time for written tasks or accept verbal submissions.
- Provide pre-printed notes so they do not have to copy notes down.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.

Sensory Processing Disorder

Inclusion Toolkit

Symptoms

- Over or under-sensitive to things like sounds, lights, touch, and smells.
- Difficulty filtering background noise.
- Emotional dysregulation when overwhelmed.

Age & Gender

- Often identified in early childhood.
- Diagnosed more often in males.
- Females are often missed due to masking.

Common myths

- It’s not bad behaviour. Sensory input is genuinely overwhelming.
- Not the same as autism, though they often co-occur.
- Not something people can overcome just by trying harder.

Diagnosis

- It isn't a separate diagnosis in the Diagnostic and Statistical Manual of Mental Disorders, the main book doctors use to diagnose conditions.
- Often identified by an occupational therapist.
- Frequently co-occurs with other conditions.

Professional support

- Sensory integration therapy with an Occupational Therapist.
- A personalised 'sensory diet' of daily activities.
- Changes to the surroundings to reduce things like noise and bright light.

How to help

- Offer quiet or low-stimulation workspaces.
- Reduce sensory triggers like flickering lights.
- Allow headphones, sunglasses, or fidget tools.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.

Auditory Processing Disorder

Inclusion Toolkit

Symptoms

- Difficulty understanding speech in noisy environments.
- Mishearing words, even with normal hearing ability.
- Needing instructions repeated or written down.

Age & Gender

- Often identified in childhood, but can persist into adulthood.
- Diagnosed more often in males.
- Females are often missed due to masking.

Common myths

- It's not a problem with the ears.
- It's not the same as ADHD, though they often co-occur.
- It's not a sign of low intelligence or inattention.

Diagnosis

- Diagnosed by an audiologist, not a GP.
- Tests measure how the brain processes sound.
- Often identified alongside dyslexia or ADHD.

Professional support

- Auditory training exercises to improve processing.
- Environmental changes to reduce background noise.
- Assistive technology such as live captions or remote microphones.

How to help

- Face the person when speaking, and speak clearly and slowly.
- Reduce background noise in meetings where possible.
- Provide written follow-ups to verbal instructions.

This information isn't a diagnostic tool. If you recognise these symptoms in yourself or others, please contact a doctor.